

ASHAMitra — One-Page Summary · AIIMS Kalyani

An offline-first, Bengali-first digital workspace for ASHA workers — built around a clinical rulebook *your committee owns*. It replaces 15+ paper registers with one app on the ASHA's own phone. The clinical core is a **deterministic, guideline-sourced rule engine — not a black-box AI** — turning answers + vitals into a referral band:  home care ·  refer PHC ≤24 h ·  refer FRU/SNCU/DH now (call 108).

Why a clinician can trust it

- **You own the rulebook.** Every decision is a readable rule citing a published guideline; the app **refuses to load rules your committee hasn't signed off**.
- **No generative AI in diagnosis.** LLMs only do Bengali voice/scribe + a general Q&A helper; the **deterministic engine makes every band decision**. Offline mode uses zero AI.
- **Reproducible & auditable.** Same input → same output; a SHA-256 protocol-hash proves which rule version decided any past case (MDSR-ready).
- **Sensitivity-first by design.** It over-refers on purpose — a missed neonatal sepsis is irreversible; a false alarm is a recoverable cost.

The seven modules

 Newborn (0–28 d) ·  Child (2 mo–5 y) ·  Pregnancy/ANC ·  Postpartum (0–42 d) ·  Immunisation (UIP) ·  Development ·  Emergency. **Sources:** IMNCI · HBNC · HBYC · PMSMA HRP · WHO ANC 2016 · UIP — *each rule cites its source*. **Engine v2.6.0:** 68 questions · 42 hard-stop · 19 combination · 24 yellow · 42 numeric rules · **64 rules awaiting your sign-off.**

How it helps

Visit notes ~30 min → ~5 min (voice + structured) · monthly HMIS report auto-generated · high-risk-pregnancy score at every ANC visit · referral outcomes tracked — closing the community-to-facility "black box" · daily immunisation-defaulter list · SAM/MAM caught at the village.

Data & security

Passwordless **phone OTP** → **role-scoped signed token** · **TLS** in transit · **PII stays on-device**, only de-identified data syncs · server-side API keys · pilot only after **AIIMS IEC clearance**, **DPDP Act 2023**-aligned.

What we ask of AIIMS Kalyani

1. **Review** the 7 MVP modules against IMNCI / HBNC / HBYC / PMSMA / WHO ANC — tell us what to change.
2. **Sign off** clinical rules before they go live (no rule reaches the field without committee approval).
3. **Oversee** a 4–6 month pilot (~20 ASHAs, 1 district near Kalyani): monthly review + open emergency-consult channel.
4. **Guide** Phase-2 priorities (TB · NCD · mental health · MUAC · RKSK) and the Phase-3 diabetic-retinopathy hand-off.

Honest note: it is a screening / decision-support tool, not a diagnostic device; the over-referral rate will be measured during the pilot; production hardening (at-rest DB encryption, enforced real-SMS OTP) completes before scale — under your oversight.

Flint De Orient Marketing & Technology Pvt Ltd (DPIIT-recognised startup) · **Sabir Ali Mollah, Founder** · Kolkata *Companion documents available: clinical modules & rules · the 11-step decision pipeline · the 64 sign-off rules · full clinical & technical briefing.*